

Pharmaniaga Primaquine Tablet 7.5mg

DESCRIPTION

Brown, round, film-coated tablet with 'RMB' breakline on one side and plain on the other.

COMPOSITION

Each film-coated tablet contains Primaquine phosphate equivalent to Primaquine 7.5 mg.

ACTIONS

Primaquine is a tissue schizontocide. The precise mechanism of action has not been determined, but may be based on primaquine's ability to bind to and alter the properties of DNA.

Primaquine is highly active against the exo-erythrocytic stages of *P. vivax* and *P. ovale* and against the primary exo-erythrocytic stages of *P. falciparum*. It is also highly active against the sexual forms of *P. falciparum*, disrupting transmission of the disease by eliminating the reservoir from which the mosquito carrier is infected.

Absorption	:	Rapidly absorbed, bioavailability is approximately 96%.
Distribution	:	Extensively distributed; unlike the 4-aminoquinoline antimalarials, there is no evidence of accumulation of primaquine or carboxyprimaquine in blood cells; the whole blood to plasma distribution ratio was 0.93 in one study of patients being treated with 15 mg (base) daily for 14 days.
Biotransformation	:	Apparent vol ₀ = Mean 248 L (range 149 - 303 L) Rapidly converted to carboxyprimaquine, the principal plasma metabolite. It is not known whether this metabolite has antimalarial activity.
Half-life	:	Primaquine - Mean, 5.8 hours (range, 3.7 to 7.4 hours).
Time to peak concentration	:	Carboxyprimaquine- 22 to 30 hours. Primaquine - Approximately 2 to 3 hours.
Peak serum concentration	:	Carboxyprimaquine – Approximately 7 hours (range, 2.6 to 8) Primaquine (steady state) - 15 mg (base) : 50- 66 nanograms per mL 30 mg (base) : Approximately 104 nanograms per mL. Carboxyprimaquine - 15 mg (base) single dose: 291-736 nanograms per mL. 15 mg (base) daily, on day 14: 432 - 1240 nanograms per mL.

Elimination	:	Less than 2% of oral primaquine dose excreted in urine within 24 hours.
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INDICATIONS

Primaquine is indicated for the prevention of relapses (radical cure) of malaria caused by *Plasmodium vivax* (and *P. ovale*). Primaquine is also effective against the gametocytes of *P. falciparum*.

CONTRAINDICATIONS

Should not be given to patients who are sensitive to primaquine and iodoquinol (a chemically related 8-quinoline).

Primaquine is contraindicated in acutely ill patients, those suffering from blood disorders, and those concurrently receiving potentially haemolytic drugs.

Primaquine is not recommended during pregnancy because primaquine may cross the placenta to a G6PD-deficient foetus and cause haemolytic anaemia in utero.

ADVERSE REACTIONS

Primaquine is generally well tolerated at normal therapeutic doses. Adverse effects include abdominal cramps, gastric distress, nausea and vomiting, headache, visual disturbances, and pruritus. Methaemoglobinaemia, mild anaemia, and leucocytosis may occasionally occur. Primaquine may rarely produce leucopenia or agranulocytosis, usually following overdosage. Hypertension and cardiac arrhythmias have been reported. Acute, dose-dependent haemolytic anaemia may occur in persons with deficiency of glucose-6-phosphate dehydrogenase (G6PD), particularly those of Mediterranean origin.

PRECAUTIONS/ WARNINGS

Primaquine should be administered cautiously to acutely ill patients with any serious systemic disease characterised by a tendency to granulocytopenia. In addition to producing haemolytic anaemia in patients with glucose-6-phosphate dehydrogenase deficiency, primaquine can produce blood dyscrasias in patients with other erythrocyte enzyme defects such as methaemoglobinaemia in NADH methaemoglobin reductase deficiency. Primaquine should be withdrawn if signs of haemolysis occur and the blood count should be monitored periodically. The administration of other bone-marrow depressants or haemolytic agents should be avoided.

The metabolism of primaquine is diminished by mepacrine and its toxicity enhanced.

The following may be especially important in patient monitoring (other tests may be warranted in some patients depending on condition)

- a) Blood cell counts
- b) Haemoglobin determinations
- c) G6PD determinations prior to treatment

Use in lactation

It is not known if primaquine is excreted in breast milk. However, primaquine has not been reported to cause problems in nursing babies.

DRUG INTERACTIONS

The following drug interactions and/or related problems have been selected on the basis of their potential clinical significance (possible mechanism in parentheses where appropriate) – not necessarily inclusive.

Note : Combination containing any of the following medications, depending on the amount present, may also interact with this medication.

Bone marrow depressants (Amphotericin B, systemic, Antithyroid agents, Azathioprine, Busulfan, Carboplatin, Carmustine, Chloramphenicol, Chromic phosphate P32, Cisplatin, Colchicine, Cyclophosphamide, Cytarabine, Dacarbazine, Dactinomycin, Daunorubicin, Doxorubicin, Etoposide, Floxuridine, Flucytosine, Flurouracil, systemic, Hydroxyurea, Ifosfamide, Interferon, Lomustine, Mechlorethamine, systemic, Melphalan, Mercaptopurine, Methotrexate, Mitomycin, Mitoxantrone, Pentamidine, Plicamycin, Procarbazine, Sodium Iodide I 131, Sodium phosphate P 32, Streptozocin, Thioguanine, Thiotepa, Uracil mustard, Vinblastine, Vincristine, Zidovudine)

(Concurrent use of primaquine with bone marrow depressants may increase the leukopenic effects; if concurrent use is required, close observation for myelotoxic effects should be considered)

Haemolytic, other (Acetohydroxamic acid, Antidiabetic agents, oral, Doxapram, Methyldopa, Nitrofurans, Primaquine, Procainamine, Quinidine, Quinine, Sulfonamides, systemic, Sulfones, Vitamin K)

(Concurrent use of primaquine with other haemolytics may increase the potential for toxic side effects)

Quinacrine
(Concurrent use is not recommended it may increase the toxic effects of primaquine)

DOSAGE AND ADMINISTRATION

A course of treatment with a schizontocide such as chloroquine is usually given to kill any erythrocytic parasites and this is given with or followed by a course of 14 daily doses of primaquine phosphate, each equivalent to 15 mg of the base, to kill the tissue forms.

Some *P. vivax* infections acquired in the south-west Pacific (the Chesson strain) may have some resistance to primaquine, and courses of treatment for up to 21 days have been recommended.

Alternative regimens of 45 mg once every 7 days for 8 weeks or 30 mg once every 7 days for 15 weeks have been suggested to minimise haemolysis in patients with glucose-6-phosphate dehydrogenase deficiency.

A course of primaquine has also been given after prolonged exposure to *P. vivax* or *P. ovale* infection to prevent a delayed primary attack in travellers returning from a highly endemic area.

A suggested dose for children is 250 to 300 µg per kg body-weight daily for 14 days.

OVERDOSAGE*Symptoms:*

Extended and severe effects of the adverse reactions.

Treatment:

There is no specific antidote and treatment must therefore, be symptomatic.

Methaemoglobinaemia may be treated by giving 200 mg ascorbic acid 3 times daily after withdrawing primaquine.

ROUTE OF ADMINISTRATION

Oral.

STORAGE CONDITIONS

Store below 30°C.

Protect from light.

Keep container tightly closed.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In bottle of 28 tablets.

In bottle of 100 tablets (for KKM tender only).

Date of revision: 12th August 2025

PRODUCT REGISTRATION HOLDER/**MANUFACTURER:**

PHARMANIAGA MANUFACTURING BERHAD (198001006232)

No. 11A, Jalan P/1, Kawasan Perusahaan Bangi,

43650 Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia.

PRP 0139.9 120825